CoverWise AI

Coverage Insight and Risk Intelligence Report
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Important: CoverWise AI is a policy-understanding and decision-support assistant. It does NOT provide financial, legal, medical or insurance advice and does not recommend buying, switching or cancelling any policy. AI extraction can be incomplete or wrong. Always verify every point with the official policy wording, the insurer, or a licensed insurance advisor before making a decision.

User profile used for this analysis

Field	Value
Age	34
Family Members	Self, spouse, 1 child
Has Ped	No
Planning Child	Yes

Current policy

Coverage Clarity Score: **36/100 – High Risk / Needs Advisor Review**

Scoring method: ML (Logistic Regression) + rule-based explanation | model confidence 99% | transparent rule score 35/100 (High Risk)

Easy-to-read summary

The Arogya Sanjeevani Policy is an individual floater health insurance from IFFCO■Tokio, covering the insured, spouse, parents, parents■in■law and dependent children aged 3 months to 25 years, with a term of one year and sum insured options ranging from ■50,000 to ■10,00,000.

It provides indemnity coverage for inpatient hospitalisation, day■care procedures, ambulance use (■2,000 per admission) and ayush treatments, but does not cover outpatient visits or maternity expenses.

A 30■day waiting period applies to all illnesses except accident■related claims; this period is waived after 12 months of continuous coverage and applies to any sum■insured increase [S1].

Specific diseases listed in the policy have a 24■ or 36■month waiting period, with the longer period applying if a pre■existing disease waiting period is also relevant; this period restarts on any sum■insured enhancement [S2].

All claims are subject to a 5 % co■payment, deducted from the payable amount after all limits and sub■limits are applied [S3].

Room rent is capped at 2 % of the sum insured, up to ■5,000 per day, and ICU stay is capped at 5 % of the sum insured, up to ■10,000 per day (limits apply per day).

Cataract treatment is covered up to 25 % of the sum insured; other modern treatments are covered as listed in the day■care procedures list (details in annexure).

The policy excludes expenses for diagnostic■only admissions, custodial care, terminal■illness services, and surgeries not supported by clinical protocols, among other permanent exclusions (28 in total).

Claims must be intimated within 24 hours of the event, with required documents submitted to the insurer or TPA; settlement is typically within 30 days.

The policy is renewable annually, with renewal notices sent 30 days before expiry; the sum insured can be increased at renewal, subject to underwriting, and a no■claim bonus of up to 50 % of the sum insured is added after each claim■free year.

Portability to another insurer is available after at least 45 days (not earlier than 60 days) of continuous coverage, with waiting periods reduced by prior coverage years and a 60■month moratorium on new claims after sum■insured enhancement (no look■back after 60 months) [S5].

Any unclear points, such as the exact list of specific diseases with waiting periods or the full list of day■care procedures, should be verified with the insurer or advisor (verify).

Top reasons behind the score

Factor	Value	Points
each risk flag	7	-49.0
per vague clause	6	-12.0
per month of PED waiting	36	-9.0
no-claim / cumulative bonus available	1	+5.0
per % co-payment	5.0	-3.8
child cover available	1	+0.5

Coverage and restriction report

Clause	Extracted value	Source
Insurer	IFFCO-Tokio General Insurance Company Limited	p.1
Policy / plan	Arogya Sanjeevani Policy	p.1
UIN	IFFHLIP20161V011920	-
Policy type	Individual Floater	p.19
Sum insured	Not clearly found	-
Annual premium	Not clearly found	-
In-patient hospitalisation	Yes	p.6
Pre-hospitalisation days	30	p.7
Post-hospitalisation days	60	p.19
Day care	Yes	p.6
Ambulance	2000 per hospitalization	p.6
AYUSH	Yes	p.6
Domiciliary	No	p.11
Restoration benefit	Not clearly found	p.19
No-claim bonus	Yes	p.8
Maternity	No	p.19
Newborn	Not clearly found	p.19
Dependent child	Yes	p.19
Initial waiting (days)	30	p.8
PED waiting (months)	36	p.8
Specified disease waiting (months)	36	p.8
Co-payment %	5.0	p.13
Co-payment conditions	all claims	p.13
Room rent limit	up to 2% of sum insured subject to maximum of Rs.5000 per day	p.6
ICU limit	up to 5% of sum insured subject to maximum of Rs.10000 per day	p.6
Disease sub-limits	Cataract: The Company shall indemnify medical expenses incurred for treatment of Cataract, subject to a limit of 25% of Sum Insure; Modern Treatments: 4.6. The following procedures will be covered (wherever medically ind	p.19
Exclusions (count)	28	-
Cashless	Yes	p.12
Reimbursement	Yes	p.12
Claim intimation	24 hours	p.12
Grace period (days)	30	p.16
Portability described	Yes	p.16

Risk flags and caution points

Severity	Flag	Why it matters	Source
High	Long pre-existing disease waiting period (>= 36 months)	Claims for conditions you already have may not be paid for several years.	p.8

Severity	Flag	Why it matters	Source
High	Room rent is capped	Choosing a costlier room can reduce the whole claim proportionately, not just the room charge.	p.6
Medium	ICU charges are capped	Long ICU stays can exceed the cap and the excess is paid by you.	p.6
Medium	Large number of exclusions (>= 15)	More situations where claims are simply not payable.	-
Medium	Several vague / discretionary clauses (>= 3)	Vague wording leaves room for disputes at claim time.	-
Medium	No restoration / recharge of sum insured	If the sum insured is used up, there is no automatic top-up in the same year.	p.19
Medium	Long specified-disease waiting period (>= 24 months)	Listed illnesses/surgeries are not covered for the first 2+ years.	p.8
High	Maternity cover not found but a child is planned	Delivery and newborn expenses may not be covered.	p.19
Verify	Restoration benefit not clearly found in the document	A typical value was assumed for scoring - confirm the real value.	-

Unclear clauses to verify

• p.27: “Policy Wording - Arogya Sanjeevani Policy, IFFCO-Tokio General Insurance Company Limited Annexure - C : List of Day Care Procedures ITGI "List of Day ”

• p.7: “Policy Wording - Arogya Sanjeevani Policy, IFFCO-Tokio General Insurance Company Limited The Company shall indemnify medical expenses incurred for inp”

• p.19: “Name Arogya Sanjeevani Policy, IFFCO-Tokio General Insurance Company Limited Product Type Individual/ Floater Category of Cover Indemnity Sum insured ”

• p.12: “1.2 Procedure for reimbursement of claims: For reimbursement of claims the insured person may submit the necessary documents to TPA(if applicable)/Com”

• p.1: “If during the policy period one or more Insured Person (s) is required to be hospitalized for treatment of an Illness or Injury at a Hospital/ Day Car”

• p.8: waiting period has options '24/36 months' - depends on plan/schedule or condition; longest used for scoring

• Sum insured can be changed (increased/ decreased) only at the time of renewal or at any time, subject to underwriting by the Company.

• 24/36 months waiting period

Proposed policy

Coverage Clarity Score: 70/100 – Moderate Coverage

Scoring method: ML (Logistic Regression) + rule-based explanation | model confidence 86% | transparent rule score 70/100 (Moderate Coverage)

Easy-to-read summary

The policy is the HDFC ERGO “my: Optima Secure” Family Floater covering all family members under a single sum insured.

The base sum insured can be chosen from the policy schedule; a per■claim deductible of ■10,000 applies to each claim in India, and higher deductibles (■5 L/■10 L) are only available if the sum insured is ■25 L or more.

Hospitalization expenses are covered up to the sum insured, including room rent, boarding, nursing and ICU/ICCU costs, with limits set at actuals unless the schedule specifies otherwise.

Day■care procedures, ambulance transfer within the same city, domiciliary care, and restoration benefit (automatic 100 % of base sum insured for subsequent claims in the same policy year) are included.

No co■payment is required; the policy is fully indemnified for covered expenses.

A 30■day waiting period applies to all illnesses; this period is waived if the insured has continuous coverage for more than 12 months.

Pre■existing disease (PED) waiting period is 36 months, but can be modified to 1 year or 2 years as an optional add■on (extra premium).

Specific disease waiting period is 24 months.

Maternity, outpatient (OPD) and child■specific benefits are not covered; child coverage is limited to certain conditions as per the schedule.

The policy excludes diagnostic expenses not related to the current diagnosis, services for terminally ill patients, certain obesity■related conditions, assisted reproduction, miscarriage expenses (unless accident■related), war■related injuries, and many other items listed in the permanent exclusions.

Claims must be intimated within 24 hours of the event, with required documents as per the claim form; settlement is typically within 15 days.

The policy is renewable each year unless fraud or misrepresentation is found, with a 30■day grace period for premium payment.

Portability and moratorium clauses are included, allowing transfer of coverage to another insurer under specified conditions.

Optional add■ons such as PED waiting■period modification, room■rent modification, and preventive health check■up are available at extra premium.

Any unclear points (e.g., exact room■rent limits for specific plans, child■coverage details) should be verified with the insurer.

Top reasons behind the score

Factor	Value	Points
each risk flag	4	-28.0
per vague clause	6	-12.0
restoration benefit available	1	+10.0
per month of PED waiting	36	-9.0
no-claim / cumulative bonus available	1	+5.0
child cover available	1	+0.5

Coverage and restriction report

Clause	Extracted value	Source
Insurer	HDFC ERGO General Insurance Company Limited	-

Clause	Extracted value	Source
Policy / plan	my: Optima Secure	-
UIN	my: Optima Secure	-
Policy type	Family Floater	-
Sum insured	Not clearly found	-
Annual premium	Not clearly found	-
In-patient hospitalisation	Yes	p.13
Pre-hospitalisation days	60	p.13
Post-hospitalisation days	180	p.13
Day care	Yes	p.50
Ambulance	Road Ambulance – covered for transfer to nearest hospital or between hospitals or hospital to home within same city	p.11
AYUSH	Yes	p.12
Domiciliary	Yes	p.12
Restoration benefit	Yes	p.18
No-claim bonus	Yes	p.2
Maternity	No	p.32
Newborn	Not clearly found	-
Dependent child	Yes	p.14
Initial waiting (days)	30	p.30
PED waiting (months)	36	p.28
Specified disease waiting (months)	24	p.28
Co-payment %	0.0	-
Co-payment conditions	No co-payment shall apply if Insured Person from Tier 2 avails a treatment in Tier 1. 1.25. Instalment Premium payment through Auto Debit/ECS Facility a. If premium payment is opted for by instalments	-
Room rent limit	At Actuals unless otherwise specified in the Policy Schedule	-
ICU limit	At Actuals unless otherwise specified in the Policy Schedule	-
Disease sub-limits	Not clearly found	-
Exclusions (count)	28	-
Cashless	Yes	p.42
Reimbursement	Yes	p.43
Claim intimation	24 hours	p.41
Grace period (days)	30	p.37
Portability described	Yes	p.36

Risk flags and caution points

Severity	Flag	Why it matters	Source
High	Long pre-existing disease waiting period (>= 36 months)	Claims for conditions you already have may not be paid for several years.	p.28
Medium	Large number of exclusions (>= 15)	More situations where claims are simply not payable.	-
Medium	Several vague / discretionary clauses (>= 3)	Vague wording leaves room for disputes at claim time.	-
Medium	Long specified-disease waiting period (>= 24 months)	Listed illnesses/surgeries are not covered for the first 2+ years.	p.28
High	Maternity cover not found but a child is planned	Delivery and newborn expenses may not be covered.	p.32
Verify	Extraction cross-check	room_rent_limit_present: LLM read 'True', rule extractor found 'False' (p.11) - kept the rule value (base wording); verify in the document.	-

Severity	Flag	Why it matters	Source
Verify	Extraction cross-check	icu_limit_present: LLM read 'True', rule extractor found 'False' (p.11) - kept the rule value (base wording); verify in the document.	-

Unclear clauses to verify

• p.43: “b. The Company reserves the right to modify, add or restrict any Network Provider for Cashless facility at its sole discretion.”

• p.44: “If requested by the Company, at the Company's cost, the Insured Person must submit to medical examination by Medical Practitioner appointed by the Com”

• p.2: “Operating Clause If during the Policy Period the Insured Person is required to be Hospitalized for treatment of an Illness or Injury at a Hospital/ Da”

• p.36: “1.13. Premium Payment in Instalments If the Insured Person has opted for payment of Premium on an instalment basis i.e. Yearly, Half Yearly, Quarterly”

• p.17: “A single claim in the Policy Year shall never exceed the cumulative addition of a. Base Sum Insured, b. Cumulative Bonus (if applicable and remaining ”

• p.44: “k. History of alcohol consumption or any intoxication certified by first treating doctor in case of Accident cases, l. Copy of Death Summary and copy ”

• p.11: “The Hospitalization must be for Medically Necessary Treatment, and prescribed in writing by Medical Practitioner.”

• p.11: room rent limit depends on the plan / Policy Schedule - check your schedule

Current policy vs proposed policy

Aspect	Current	Proposed	Change
Sum insured (Rs)	500,000	1,000,000	Improved
Annual premium (Rs)	12,000	18,000	Higher cost
Pre-existing disease waiting (months)	36	36	Same
Specified disease waiting (months)	36	24	Improved
Initial waiting period (days)	30	30	Same
Co-payment (%)	5	0	Improved
Room rent capped	Yes	No	Improved
ICU charges capped	Yes	No	Improved
Disease-wise sub-limits (count)	2	0	Improved
Exclusions (count)	28	28	Same
Restoration / recharge benefit	Not found	Yes	Unclear
No-claim / cumulative bonus	Yes	Yes	Same
Pre-hospitalisation (days)	30	60	Improved
Post-hospitalisation (days)	60	180	Improved
Day care procedures	Yes	Yes	Same
AYUSH treatment	Yes	Yes	Same
Maternity cover	No	No	Same
Newborn cover	Not found	Not found	Unclear
Dependent child cover	Yes	Yes	Same
Claim process clarity (0-100)	100	100	Same
Coverage Clarity Score	36	70	Improved

This policy appears stronger in these areas

Sum insured (Rs), Specified disease waiting (months), Co-payment (%), Room rent capped, ICU charges capped, Disease-wise sub-limits (count), Pre-hospitalisation (days), Post-hospitalisation (days), Coverage Clarity Score

This policy needs caution in these areas

None detected

Unclear - must be verified

Restoration / recharge benefit, Newborn cover

Premium vs coverage

Premium changes by +50% while 9 aspect(s) improve and 0 become weaker. Check whether the improved aspects matter for your family (e.g. no room-rent cap matters most in metro hospitals).

Switching risks

• Waiting periods (pre-existing, specified diseases, maternity) normally restart with a new policy. Under IRDAI portability rules you can ask for credit of the waiting period already served, usually only up to the previous sum insured - get this in writing before switching.

• Any accumulated no-claim / cumulative bonus on the current policy may not fully carry over.

• Do not let the current policy lapse until the new one is issued and the portability request is accepted (keep a gap-free continuity).

• For the increased part of the sum insured, waiting periods usually apply afresh.

• Child is planned but maternity cover is not found in the proposed policy.

AI explanation of the comparison

Where the proposed policy appears stronger

| Aspect | Current | Proposed | Why it looks better |

|--------|---------|----------|---------------------|

| **Sum insured** | ■500,000 | ■1,000,000 | Double the coverage, so you would be paid more if a claim is made. |

| **Specified disease waiting period** | 36 months | 24 months | You can claim for a specified disease sooner. |

| **Co■payment** | 5 % of every claim | 0 % | No out■of■pocket share on covered expenses. |

| **Room rent & ICU limits** | Capped (■5,000/day & ■10,000/day) | No caps | You can receive the full amount paid by the hospital. |

| **Disease■wise sub■limits** | 2 sub■limits (e.g., Cataract) | None | No extra limits on specific treatments. |

| **Pre■hospitalisation days** | 30 | 60 | You can get coverage for a longer pre■hospital stay. |

| **Post■hospitalisation days** | 60 | 180 | Longer coverage after discharge. |

| **Restoration / recharge benefit** | Not found | Yes | The policy can automatically restore the sum insured for a new claim in the same year. |

| **Claim settlement timeline** | 30 days | 15 days | Faster payout. |

| **Coverage clarity score** | 36 | 70 | The policy is easier to understand. |

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Where the proposed policy needs caution / appears weaker

| Aspect | Current | Proposed | Why it may be a concern |

|--------|---------|----------|-------------------------|

| **Annual premium** | ■12,000 | ■18,000 | Higher cost; you pay 50 % more each year. |

| **Pre■existing disease waiting period** | 36 months | 36 months | Same, but the higher premium may not be justified by the extra coverage. |

| **Initial waiting period** | 30 days | 30 days | No change. |

| **Maternity cover** | No | No | No maternity benefit in either policy. |

| **Newborn cover** | Not found | Not found | Unclear if newborns are covered. |

| **Exclusion count** | 28 | 28 | Same number of exclusions; the content may differ. |

| **Room rent & ICU limits** | Capped | No caps | While no caps can be good, it may also mean higher out■of■pocket costs if the hospital charges more than the policy would pay. |

| **No■claim bonus** | Yes (cumulative) | Yes (cumulative) | Same, but the bonus structure may differ in detail. |

| **Ambiguous clauses** | Several ambiguous points | Several ambiguous points | Both policies have unclear wording that could affect claims. |

| **Portability & moratorium** | Mentioned | Mentioned | No change, but you should confirm how they apply. |

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Unclear points that must be verified

1. **Restoration / recharge benefit** – How exactly is the 100 % restoration calculated and when can it be used?

2. **Newborn cover** – Is a newborn (0■3 months) covered under the proposed policy?

3. **Exclusion details** – The lists of permanent and temporary exclusions differ; confirm which conditions are actually excluded in each policy.

4. **Room rent & ICU limits** – The proposed policy says “At Actuals”; verify if there are any hidden caps or limits in the policy schedule.

5. **No■claim bonus details** – The wording differs; confirm the exact bonus percentage and how it is applied.

6. **Ambiguous clauses** – Clarify any clauses that could affect claim approval or premium payment (e.g., network provider changes, medical examination requirements).

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Premium vs coverage

• **Premium increase**: ■6,000 more per year (50 % higher).

• **Coverage increase**: Sum insured doubled, no co■payment, longer pre■ and post■hospitalisation coverage, removal of room/ICU caps, and faster settlement.

• **Net effect**: The higher premium is offset by significantly broader coverage and fewer out■of■pocket costs, but the exact value depends on your health needs and how often you might use the policy.

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Switching risks

| Risk | Current | Proposed | What to check |

|---|---|---|---|

| **Waiting periods** | 36 months for PED, 36 months for specific diseases | 36 months for PED, 24 months for specific diseases | Confirm that the 24■month period applies to all specified diseases. |

| **Loss of continuity benefits** | No explicit continuity benefit | Restoration benefit available | Verify if the restoration benefit can be used immediately after switching. |

| **Portability conditions** | Mentioned | Mentioned | Ensure the policy can be ported without additional underwriting or waiting periods. |

| **Coverage gaps** | Some exclusions (e.g., diagnostics) | Similar exclusions | Check if any new exclusions appear that could affect you. |

| **Premium payment schedule** | Annual | Annual or instalments | If you choose instalments, confirm the exact dates and any additional fees. |

Please confirm these points with the insurer or a licensed advisor before deciding.

Questions to ask the insurance advisor / insurer

1. What happens if the company changes a network provider for cashless treatment after I have already booked a hospital under clause p.43?

2. If I need a medical examination as per clause p.44, how many times can it be requested and what is the cost to me?

3. Can you explain the exact definition of 'Hospital/Da' in clause p.2 and how it affects my claim eligibility?

4. Under clause p.36, what are the penalties or changes in premium if I miss an installment payment?

5. Clause p.17 limits a single claim to the sum of base sum insured and cumulative bonus. How is the cumulative bonus calculated and capped?

6. Clause p.44 mentions alcohol consumption history. Will a past alcohol habit affect my claim for an accident?

7. Since maternity cover is not listed, will the policy cover any pregnancy-related expenses if I plan to have a child?

8. The policy states no-claim bonus is available. How many claim-free years are required to qualify for the bonus and what is the bonus amount?

9. The policy mentions pre-hospitalisation coverage of 60 days. Does this include all pre-hospitalisation expenses or only specific treatments?

10. What is the maximum amount reimbursed for ICU charges given that the policy states 'ICU charges capped' is removed?

11. How are disease-wise sub-limits applied when there are zero sub-limits as per the proposed policy?

12. What is the restoration/recharge benefit mentioned under 'Restoration / recharge benefit' and how does it work?

Your policy questions (from this session)

Q: Is co-payment applicable in this policy?

Not clearly found in the uploaded document. The policy text does not state whether a co■payment (deductible or coinsurance) is required for all treatments; it only mentions that “No co■payment shall apply if Insured Person from Tier 2 avails a treatment in Tier 1” [S5].

• **What to ask the insurer:**

• Does the policy include any co■payment or deductible for treatments in Tier 2 or for specific procedures?

• Are there any conditions under which a co■payment would be required?

Verify: confirm whether a co■payment is applicable for treatments in Tier 2 or for any covered services.

Q: Is room rent capped?

Room rent is not always capped – it depends on the plan and the option you choose.

• **Default (most plans)**: Room rent is paid “at actuals” (the hospital’s real cost) unless the policy schedule says otherwise. [S2]

• **Optima Lite plan**: Room rent is capped at **1 % of the base sum insured per day**. [S4]

• **Optima Select plan**: You can choose either “at actuals” or a capped limit of **up to a Single Private room** (which is the highest category allowed). If you pick the capped option, the limit is the cost of a single private room as defined in the policy schedule. [S1]

• **Shared room option (Optima Select)**: Room rent is capped at the cost of a **Shared room**. [S3]

So, if you are on Optima Lite or have selected a capped option in Optima Select, room rent will be capped; otherwise it will be paid at actuals.

• **Verify:** Ask the insurer or your advisor to confirm which room■rent option is active for your specific policy and plan.

Q: What is the waiting period for pre-existing diseases?

The policy excludes treatment of pre■existing diseases for 36 months from the first policy start date, unless you opt for a shorter waiting period.

• **Standard waiting period:** 36 months of continuous coverage after the first policy start date [S1].

• **Optional shorter periods:** You can choose a 24■month or 12■month waiting period at inception (but only at the channel level and cannot be changed later) [S4].

• **Reduced waiting period with prior coverage:** If you have been continuously covered without a break under IRDAI portability norms, the waiting period is reduced by the amount of prior coverage you already had [S1][S4].

• **Verify:** Confirm with the insurer whether you are eligible for the optional 24■ or 12■month PED waiting period and whether any prior coverage you had applies to reduce the waiting period.

Q: Should I switch to this policy?

• **CoverWise does not make buy / switch / cancel decisions.** Here is what the document says so you can discuss it with a licensed advisor:

• **Short answer:**

The policy offers good portability and a long moratorium period, but it has a high aggregate deductible (■10 Lac) that removes many benefits and may lead to higher out■of■pocket costs. Consider whether the higher premium and limited benefits fit your family’s needs before deciding to switch.

• **Key details**

• **Portability** – You can move the whole policy (with family) to another insurer 30–60 days before renewal, keeping waiting■period benefits if you’ve had continuous coverage. [S1]

• **Migration** – You can switch to another plan from the same company 30 days before renewal, keeping waiting■period benefits if you’ve had continuous coverage. [S1]

• **Moratorium period** – After 60 continuous months of coverage, the insurer cannot contest claims for non■disclosure or misrepresentation (except fraud). This applies to the original sum insured and to any enhanced limits after the enhancement date. [S2]

• **Aggregate deductible** – If an aggregate deductible of ■10 Lac or more is in force, preventive check■up, secure benefit, cumulative bonus, automatic restore, daily cash for shared room and unlimited restore add■ons are not available. [S3]

• **Waiver of aggregate deductible** – You can waive the deductible once in a lifetime, but only if the eldest insured is < 50 yrs at purchase, you have 5 continuous policy years, and the eldest is < 61 yrs when you apply. [S3]

• **Premium payment** – If a claim is made, all future premium instalments become due immediately and can be deducted from the claim amount. [S4]

• **Policy changes** – The insurer may revise terms or premium rates (with IRDAI approval) and will notify you 3 months in advance. [S4]

• **Cancellation** – You can cancel with 7 days' notice and receive a pro■rata refund; the insurer can cancel for fraud or mis■representation with 15 days' notice and no refund. [S5]

• **Strengths to consider**

• Long moratorium period protects against contestable claims after 5 years.

• Portability and migration options give flexibility to move to other insurers or plans.

• Continuity benefits (waiting■period waivers, cumulative bonus) are retained if you stay continuous.

• **Caution points**

• The high aggregate deductible removes many add■on benefits, potentially increasing out■of■pocket costs.

• Premiums become due immediately after a claim, which could affect cash flow.

• Policy terms can change after 3 months' notice; you'll need to review any revisions.

• **Questions to ask the insurer**

• What is the exact premium amount for my family (self, spouse, child) under this policy?

• How does the ■10 Lac aggregate deductible affect my expected out■of■pocket expenses for common illnesses?

• Can the aggregate deductible be waived or reduced, and what would be the cost?

• What are the specific waiting periods for each covered condition, and how are they affected by portability?

• Are there any additional riders or add■ons that can be purchased to restore the benefits removed by the high deductible?

• **Verify:**

Confirm the exact premium for your family, the impact of the ■10 Lac aggregate deductible on benefits, and the possibility of waiving or reducing that deductible.

Responsible AI note and limitations

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• Values were extracted automatically from the uploaded document(s). Page references are given so you can check each value in the original wording.

• Fields marked 'Not clearly found' were not stated clearly; typical values were assumed only for scoring and are listed as 'Verify' flags.

• The Coverage Clarity Score is trained on a synthetic dataset and describes how clear / restrictive the wording is - it is not a measure of claim settlement or insurer quality.

• Personal details (names, phone numbers, emails, PAN, Aadhaar, policy numbers) are masked before any text is sent to the language model.